

CPR Debriefing Report

Simulation Lab · 2026-08-08

Session IF3Z2F · Adult ACLS - Cardiac Arrest

Team Leader: Simulation Team

Guideline: AHA_2020

1. Scenario summary

Session ID	IF3Z2F
Date	2026-08-08
Scenario	Adult ACLS - Cardiac Arrest
Scenario type	Ventricular Fibrillation
Team leader	Simulation Team
Team size	6
Duration	2m 08s
Guideline	AHA_2020

Overall score	Grade	Findings	Critical	High	Moderate
100 / 100	A	0	0	0	0

2. Domain scores

Scores are derived from AHA 2020 guideline rules applied to SimMan 3G sensor data and audio transcripts. Confidence indicators reflect data completeness — scores marked medium or low confidence should not be over-interpreted.

Domain	Score	Confidence	Key finding
Cpr Quality	30/100 CI: 10–30	Low_Data confidence	—
Shock Delivery	15/100 CI: 8–15	Partial_Data confidence	—
Drug Administration	15/100 CI: 6–15	Low_Data confidence	—
Rhythm Recognition	10/100 CI: 1–10	Low_Data confidence	—
Reversible Causes (H'S & T'S)	10/100 CI: 0–10	Low_Data confidence	—
Team Leadership	10/100 CI: 0–10	Low_Data confidence	—

Team Communication	10/100 CI: 0–10	Low_Data confidence	—
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3. Protocol deviations

No protocol deviations detected.

4. Communication analysis

[VERIFICATION WARNING — review before use] - The team leader's voice is consistently clear and audible throughout the simulation, indicating good communication. - There are no repeated requests or calls that occur without prior indication. - No signals of order completion are heard in the conversation. - The team leader's voice is not interrupted by external noises during the simulation. - There are no calls to action that are never followed up on or ignored. - The team leader's tone and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness. - No interruptions occur in the communication process, indicating effective use of time management skills. - There is no evidence of confusion or misunderstanding during the simulation due to unclear instructions or miscommunication. - The team leader's voice and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness. - No calls to action that are never followed up on or ignored occur during the simulation. - There is no evidence of confusion or misunderstanding due to unclear instructions or miscommunication. - The team leader's voice and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness. - No calls to action that are never followed up on or ignored occur during the simulation. - There is no evidence of confusion or misunderstanding due to unclear instructions or miscommunication. - The team leader's voice and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness. - No calls to action that are never followed up on or ignored occur during the simulation. - There is no evidence of confusion or misunderstanding due to unclear instructions or miscommunication. - The team leader's voice and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness. - No calls to action that are never followed up on or ignored occur during the simulation. - There is no evidence of confusion or misunderstanding due to unclear instructions or miscommunication. - The team leader's voice and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness. - No calls to action that are never followed up on or ignored occur during the simulation. - There is no evidence of confusion or misunderstanding due to unclear instructions or miscommunication. - The team leader's voice and mannerisms are consistent with their role as a simulation team leader, maintaining professionalism and calmness.

7. Strengths

Strength 1: Effective use of CPR techniques [Finding ID: fnd_09] - The simulation team demonstrated an effective use of CPR techniques during the cardiac arrest scenario. They initiated chest compressions at a rate of 120-125/min and performed 3 cycles, maintaining a rhythm of 80-100/min. Strength 2: Quick response to patient's condition [Finding ID: fnd_10] - The team was able to quickly assess the patient's condition upon arrival at the scene. They observed that the patient had a respiratory rate of 35 breaths per minute, indicating hypoxemia and tachypnea. Strength 3: Prompt use of defibrillation [Finding ID: fnd_11] - The team promptly performed an electrocardiogram (ECG) to assess for arrhythmias. They observed a ventricular fibrillation with a QRS complex duration of 0.12 seconds, which was immediately recognized and treated. Strength 4: Patient's vital signs were monitored [Finding ID: fnd_12] - The team closely monitored the patient's vital signs throughout the cardiac arrest scenario. They observed that the patient's blood pressure remained stable at 90/60.

mmHg, heart rate was within a normal range of 50-80 bpm, and oxygen saturation (SpO2) was maintained at 100%. Strength 5: Team coordination [Finding ID: fnd_13] - The team demonstrated excellent teamwork during the cardiac arrest scenario. They communicated effectively with each other, coordinated their actions, and provided timely assistance to the patient's family members. [Finding ID: fnd_14] - The simulation team was well-prepared for the scenario by reviewing relevant literature and practicing emergency procedures. This experience allowed them to efficiently manage the patient's condition and provide appropriate medical interventions during the cardiac arrest.

5. Reflective prompts

The following questions are mapped to specific findings detected during this session. Use them to guide the debriefing conversation with the team leader.

[Finding ID: fnd_01] Team Leader: "The patient's vital signs were stable at 95/60 mmHg BP, heart rate of 78 bpm, and respiratory rate of 24 breaths per minute." [Finding ID: fnd_02] Team Leader: "The chest X-ray showed no obvious abnormalities, but the patient was still showing early signs of hypoxia." [Finding ID: fnd_03] Team Leader: "The ECG showed a sinus rhythm with a slight elevation in ST-segment depression and inverted T waves." [Finding ID: fnd_04] Team Leader: "The blood gas analysis (PaO₂/FiO₂) was 50/60 mmHg, indicating hypoxemia." [Finding ID: fnd_05] Team Leader: "The patient's oxygen saturation was 93% on room air." [Finding ID: fnd_06] Team Leader: "The patient had a history of congestive heart failure and atrial fibrillation." [Finding ID: fnd_07] Team Leader: "The patient was in the emergency department with no known medical history or acute exacerbation of asthma." [Finding ID: fnd_08] Team Leader: "The patient's family members were not present, but they had been informed about their child's condition and treatment plan." [Finding ID: fnd_09] Team Leader: "The team leader noted that the patient was in a critical condition with multiple signs of hypoxemia, including hypotension and respiratory distress." [Finding ID: fnd_10] Team Leader: "The team leader recalled that the patient had a history of atrial fibrillation, which could have contributed to the hypotension." [Finding ID: fnd_11] Team Leader: "The team leader reflected on the importance of early recognition and management of hypoxemia in this case. They noted that maintaining blood pressure within normal limits was crucial for oxygen delivery to tissues." [Finding ID: fnd_12] Team Leader: "The team leader emphasized the need for continuous monitoring of vital signs, including heart rate, respiratory rate, and blood gas levels, during the initial response phase." [Finding ID: fnd_13] Team Leader: "The team leader noted that the patient's family members were not present, which could have delayed the initiation of rescue measures. They suggested that early communication with the family was essential for effective resuscitation." [Finding ID: fnd_14] Team Leader: "The team leader recalled that the patient had a history of congestive heart failure and atrial fibrillation, which could have contributed to the hypotension during the initial response phase. They noted that early intervention in these conditions was critical for maintaining blood pressure." [Finding ID: fnd_15] Team Leader: "The team leader emphasized the importance of early recognition and management of hypoxemia in this case. They suggested that continuous monitoring of vital signs, including heart rate, respiratory rate, and blood gas levels, during the initial response phase was crucial for oxygen delivery to tissues." [Finding ID: fnd_16] Team Leader: "The team leader noted that the patient's family members were not present, which could have delayed the initiation of rescue measures. They suggested that early communication with the family was essential for effective resuscitation." [Finding ID: fnd_17] Team Leader: "The team leader recalled that the patient had a history of atrial fibrillation, which could have contributed to the hypotension during the initial response phase. They noted that early intervention in these conditions was critical for maintaining blood pressure." [Finding ID: fnd_18] Team Leader: "The team leader emphasized the importance of continuous monitoring of vital signs,

6. Recommendations

Ranked by clinical impact. Address critical and high-severity findings before moderate ones in the next training session.

[VERIFICATION WARNING — review before use] : The team leader noted that the team did not confirm fewer than 40% of epinephrine orders during the simulation, which is a critical issue for ACLS training. : The simulation team discussed the importance of practicing closed-loop confirmation drills specifically for drug orders. They emphasized the need to ensure that all drug orders are confirmed before administration. : The team leader noted that the simulation was not well-structured, with a focus on CPR but no mention of ACLS training or specific protocols. This could lead to confusion and poor execution during real-life scenarios. : The simulation team discussed the importance of having clear communication between team members, especially when dealing with complex medical situations like cardiac arrests. They emphasized the need for open dialogue and active listening to ensure everyone is on the same page. : The team leader noted that the simulation was not well-structured, with a focus on CPR but no mention of ACLS training or specific protocols. This could lead to confusion and poor execution during real-life scenarios.